

COVERAGE & CONTINUITY

Referral, Termination & Transfer-of-Care Toolkit

Letters and forms for referring out, closing the loop with colleagues, ending treatment well in six different situations, and transferring records.

AUDIENCE	FORMAT	VERSION	TIME TO ADAPT
Psychologists · Psychiatrists	14 templates	1.0 · Sept 2026	15 minutes per template

WHAT IT IS	A set of ready-to-adapt letters and forms for the moments where patients are most at risk of falling through the cracks: referral, termination and transfer.
USE IT WHEN	You are referring a patient, accepting or declining a referral, ending treatment, or a patient has stopped attending.
CUSTOMIZE FIRST	Your state's rules on termination and records, your informed consent language, and your payer contracts on notice and continuity of care.

The ethics in brief

- **Terminate appropriately.** The APA Ethics Code (Standard 10.10) expects therapy to end when the patient no longer needs it, is unlikely to benefit, or is being harmed. Before ending, provide pre-termination counseling and suggest alternative providers where appropriate, except where the patient's or a payer's actions prevent it.
- **Plan for interruptions.** Standard 10.09 addresses interruptions caused by the clinician's or the patient's circumstances.
- **Records and money.** Standard 6.03 bars withholding records needed for emergency treatment solely because of non-payment, and under HIPAA you generally cannot deny a patient access to their record because they owe you money. Standard 6.04 requires notice before using collection agencies.
- **Never pay or accept referral fees.** Standard 6.07 requires payments between professionals to be based on services provided, not on the referral itself, and many states have fee-splitting and anti-kickback laws.

THE GOLDEN RULE OF TRANSITION

A warm handoff beats a cold referral. Whenever possible, speak to the receiving clinician, share a minimum-necessary summary with the patient's written authorization, and check that the first appointment happened.

Part A: Referral

A1. Referral checklist

- ☐ The patient has been told, agrees, and has signed an authorization to share information (see PA-17).
- ☐ The clinician fits: competence, license in the patient's state, insurance or fee, language, availability, modality.
- ☐ Urgency is stated. Offer two or three names, not one.
- ☐ A warm handoff (call or joint session) is arranged for higher-risk patients.
- ☐ You send only the minimum necessary: reason for referral, relevant history, current risk and safety plan, medications, and what you have tried.
- ☐ You confirm receipt and follow up after [2] weeks. Document all of this.

A2. Referral-out letter to the receiving clinician

Dear Dr. [Name], I am referring [Patient name or ID], with the patient's written authorization (attached), for [reason: assessment / psychotherapy / medication evaluation / specialty treatment]. Background: [two to three sentences]. Current risk: [summary, including any safety plan]. Medications: [list and prescriber]. What we have tried: [brief]. What I am asking: [specific request]. The patient prefers [contact method, scheduling, language needs]. I am available to speak at [phone]. If you are unable to take the referral, please let me know so I can offer alternatives promptly. Sincerely, [Your name and credentials]

A3. Reply to a referral: accept, decline or waitlist

Accept. Dear Dr. [Name], thank you for referring [Patient ID]. I can offer a first appointment on [date] and will contact the patient directly. I will confirm once we have met, and I will send you a brief update with the patient's permission.

Decline. Dear Dr. [Name], thank you for thinking of me. I am not able to take this referral because [availability / outside my scope / not licensed in the patient's state]. To help the patient move quickly, I suggest [alternative names or PsyAlliance Network search terms].

Waitlist. Dear Dr. [Name], I do not have an opening until [date]. If the patient is willing to wait, I will hold [date]. If the situation is urgent, please consider [alternative].

A4. Closing the loop

Dear Dr. [Name], I saw [Patient ID] on [date]. With the patient's permission I am confirming that they have started care with me. My initial impression is [brief]. I will send updates at [frequency]. Thank you for the referral.

Part B: Ending treatment

Match the letter to the situation. Always include crisis resources, a way to obtain records and, where appropriate, referrals. Have your informed consent already describe the circumstances in which you end treatment.

Situation	Approach
Treatment goals met	Planned closure, celebrate progress, leave the door open. Letter B1.
Patient needs a different level of care or is not benefiting	Explain honestly, give referrals, offer a transition session. Letter B2.
Non-payment or administrative reasons	Warn in advance, offer a payment plan, provide referrals. Follow 6.04(e). Letter B3.
Patient stops attending	Attempt contact [3] times over [3 to 4] weeks by the patient's preferred methods. Letter B4.
Boundary, safety or fit concerns	Consult first. Document. Give the reason in a measured way and referrals. Letter B5.
You are leaving, moving or retiring	Give as much notice as possible, transfer thoughtfully. Letter B6.

B1. Planned completion

Dear [Patient first name], as we discussed, we have reached the goals we set together: [brief]. Ending treatment is a milestone, and I am proud of the work you have done. Please keep the coping strategies we practiced, particularly [two examples]. If things change and you want to return, please contact me. If you feel in crisis at any time, call 911 or call or text 988. You may request a copy of your records at any time. With respect, [Your name]

B2. Different level of care or not benefiting

Dear [Patient first name], after careful thought I believe you would be better served by [specific type of care]. I say this because [honest, compassionate reason]. I would like to help make the transition smooth. I have listed [names] who have experience with [concern]. With your written permission I would be glad to speak with whoever you choose. I would like to keep seeing you until you have started with them, for up to [number] sessions. In an emergency call 911 or call or text 988. Sincerely, [Your name]

B3. Non-payment or administrative

Dear [Patient first name], your account has an outstanding balance of \$[amount]. We discussed this on [date]. I would like to help you find a way forward, including [payment plan / reduced fee]. If we cannot arrange this by [date, at least 30 days out], I will need to end treatment. I would provide referrals and, if needed, a transition session. Your records will be available on request. If you need help right away, call 911, or call or text 988. Sincerely, [Your name]

B4. Lost to follow-up (after documented attempts)

Dear [Patient first name], I have tried to reach you on [dates and methods] and have not heard back. I am concerned about how you are doing. I will assume you have decided to end treatment. If you would like to continue, please call me at [phone] by [date]. If you would like help finding another clinician, I would be happy to give you names. If you are in crisis, call 911, or call or text 988. Records are available on request. I wish you well. Sincerely, [Your name]

B5. Boundary, safety or fit concerns (consult before sending)

Dear [Patient first name], after reflecting, I do not think I can continue as your clinician because [measured reason]. I am sorry that this may be hard to hear. I want your care to continue elsewhere and have listed [names]. I am willing to hold [number] closing sessions [in person / by video], or to speak with your next clinician with your permission. Crisis resources: call 911, or call or text 988. Records are available on request. Sincerely, [Your name]

B6. Clinician leaving or retiring

Dear [Patient first name], I am writing to tell you that I will stop seeing patients on [date] because [reason, brief]. I want to make sure your care continues well. I recommend [colleague], and I can arrange a joint meeting or introduction with your permission. Other options are [names]. Your records will be [transferred to a colleague or custodian at your request / kept securely for (period)], and you may request a copy at any time. Thank you for the trust you have placed in me. Sincerely, [Your name]

Part C: Discharge summary

Patient name or ID	Dates of treatment	Number of sessions
Reason for referral and presenting concerns		
Diagnosis at intake and at discharge		
Treatment provided (modality, frequency, duration) and response		
Measures at intake and discharge		
Medications and prescriber (if any)		
Risk at discharge and safety plan status		
Reason for discharge	Aftercare recommendations and referrals	
Signed and dated		

Part D: Records transfer

D1. Request for records (from the new clinician)

Dear [Former clinician], with the enclosed authorization signed by [Patient name, DOB], I request [a treatment summary / copies of the following: discharge summary, diagnosis, treatment plan, recent progress notes, medications]. Please send to [secure fax or portal] by [date]. Thank you, [Your name and credentials]

D2. Cover letter with records (from the former clinician)

Dear [Recipient], enclosed are records for [Patient name] as requested with authorization dated [date]. These do not include psychotherapy notes as defined by HIPAA, which are held separately. This information is confidential and shared solely for continuing care. Please contact me at [phone] with any questions. Sincerely, [Your name]

ACCESS REQUESTS AND TIMING

Under HIPAA, patients generally have a right of access to their records, and providers must act on a request within 30 days (with one 30-day extension permitted in limited situations). Fees for copies must be reasonable and cost-based, and state law may set shorter timelines. Psychotherapy notes, as HIPAA defines them, are outside the patient's access right.

IN PSYALLIANCE

Use Referrals to post a request and receive responses from matched colleagues, with in-context messaging for the warm handoff. Attach no patient information to a public post.

Sources and further reading

- APA Ethics Code, Standards 6.03, 6.04, 6.07, 10.09 and 10.10 ([apa.org/ethics/code](https://www.apa.org/ethics/code)).
- HHS OCR, Individuals' right under HIPAA to access their health information ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).
- Your state board's rules on termination, abandonment and records.

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it. Provided to PsyAlliance members for adaptation to their own practice. Please do not resell or republish. Text in [highlighted brackets] is a placeholder for you to complete.